MIC: A High-Throughput Lossless Codec for Medical Images

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Abstract

We present MIC (Medical Image Codec), a lossless compression codec optimized for 16-bit medical images stored in the DICOM format. MIC chains three lightweight stages—spatial delta prediction, run-length encoding (RLE), and finite state entropy (FSE) coding based on asymmetric numeral systems (ANS)—to achieve compression ratios of 1.7×–8.6× on greyscale DICOM images and 3–5× on RGB whole slide imaging (WSI) tissue tiles, while delivering decompression throughput up to 16 GB/s on a 64-core ARM64 server. The codec supports three container formats: MIC1 for single frames, MIC2 for multi-frame sequences with optional temporal prediction, and MIC3 for tiled whole slide images with multi-resolution pyramid levels. We evaluate MIC on eight clinical DICOM datasets spanning MR, CT, CR, X-ray, and mammography modalities, and compare it against a 5/3 integer wavelet alternative. Delta+RLE+FSE strictly dominates the wavelet pipeline on all tested modalities in both compression ratio and decompression speed. MIC is implemented in Go with JavaScript and WebAssembly browser decoders, and is released as open-source software.

1 Introduction

Medical imaging generates enormous volumes of data. A single digital breast tomosynthesis (DBT) study produces 69 or more frames of 2457×1890 pixels at 10-bit depth, totalling over 600 MB of raw pixel data. Whole slide pathology images routinely exceed 100 000×100 000 pixels. Efficient lossless compression is essential for archival storage, network transmission, and real-time rendering of these images in clinical picture archiving and communication systems (PACS) and diagnostic viewers.

The DICOM standard [1] supports several transfer syntaxes for lossless compression, including JPEG 2000 [2], JPEG-LS [4], and RLE Lossless. JPEG 2000 achieves strong compression ratios through its 5/3 integer wavelet transform but incurs high computational cost during decompression, which limits rendering throughput. JPEG-LS provides fast encoding and decoding but typically achieves lower compression ratios than JPEG 2000. DICOM RLE is simple but inefficient.

We introduce MIC, a codec that targets the intersection of high compression ratio and high decompression throughput. MIC exploits three properties of medical images:

1. **Spatial correlation**: Adjacent pixels are highly correlated, making a simple linear predictor highly effective.
2. **Residual sparsity**: After prediction, pixel residuals cluster tightly around zero with long runs of identical values.
3. **Moderate symbol cardinality**: Even 16-bit images typically produce fewer than 1 000 distinct residual values after delta coding.

The codec processes pixels through a three-stage pipeline: spatial delta encoding, run-length encoding, and FSE (ANS-based) entropy coding. Each stage is simple enough to decode in a single sequential pass with minimal branching, enabling decompression speeds that scale with memory bandwidth. On a 64-core AWS Graviton3 server (c7g.metal), MIC decompresses mammography images at 16 GB/s—substantially faster than JPEG 2000 decoders.

Beyond single-frame greyscale, MIC supports:

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- **Multi-frame images** (MIC2 container) with random-access frame indexing and optional inter-frame temporal prediction for video-like sequences.
- **Whole slide images** (MIC3 container) with tiled architecture, YCoCg-R reversible color transform, multi-resolution pyramid levels, and parallel tile compression.

The remainder of this paper is organized as follows. Section 2 surveys related work. Section 3 describes the compression pipeline. Section 4 details the MIC2 and MIC3 container formats. Section 5 covers implementation details and optimizations. Section 6 presents experimental results. Section 7 compares against a wavelet-based alternative. Section 8 discusses design tradeoffs and limitations. Section 9 concludes.

2 Related Work

2.1 Lossless Image Compression in DICOM

JPEG 2000 Part 1 [2] employs a 5/3 reversible integer wavelet transform with embedded block coding (EBCOT). It achieves excellent compression ratios but its bit-plane coding and context modeling impose significant computational overhead during decompression. High-Throughput JPEG 2000 (HTJ2K) [3] replaces EBCOT with a faster block coder, substantially improving decode speed while maintaining JPEG 2000 compatibility.

JPEG-LS [4] uses context-adaptive prediction (MED predictor) followed by Golomb-Rice coding. It provides fast encoding and decoding with moderate compression ratios. JPEG-LS is well-suited to smooth images but its prediction model is fixed and cannot adapt to the varying statistics of different medical modalities.

RLE Lossless, specified in the DICOM standard as Transfer Syntax 1.2.840.10008.1.2.5, uses byte-level PackBits run-length encoding. It is trivially simple but achieves poor compression ratios (typically $<1.5\times$) because it operates on bytes rather than 16-bit pixel values and lacks any decorrelation stage.

2.2 Entropy Coding

Traditional arithmetic coding achieves near-optimal compression but is inherently sequential: the data dependencies between encoder and decoder states resist parallelization. Mahapatra and Singh [5] addressed this with a parallel-pipelined FPGA implementation of multi-alphabet arithmetic coding, demonstrating that hardware acceleration can overcome the throughput limitations of sequential arithmetic coders. Asymmetric numeral systems (ANS) [6] take a different approach, generalizing arithmetic coding into a single-state machine that replaces the interval subdivision of arithmetic coding with integer state transitions. Finite State Entropy (FSE) [7] is a table-driven implementation of tANS (table-based ANS) that achieves near-optimal compression with $O(1)$ per-symbol encode and decode operations, without requiring the specialized hardware of [5]. FSE was popularized by its use in Zstandard [8] and is used in MIC for the final entropy coding stage.

Canonical Huffman coding [9] assigns codewords based on sorted code lengths, enabling compact codebook representation. MIC includes a canonical Huffman backend as an alternative to FSE; it produces slightly smaller output but decodes more slowly.

2.3 Color Transforms for Pathology Imaging

The YCoCg-R transform [10] is a reversible integer approximation of the RGB-to-YCbCr color space conversion. It decorrelates RGB channels into luminance (Y) and chrominance (Co, Cg) with no loss of precision. YCoCg-R has been adopted in screen content coding extensions to H.265/HEVC and is used in MIC for whole slide image compression.

3 Compression Pipeline

MIC processes 16-bit greyscale pixels through three sequential stages. Figure 1 illustrates the dataflow.

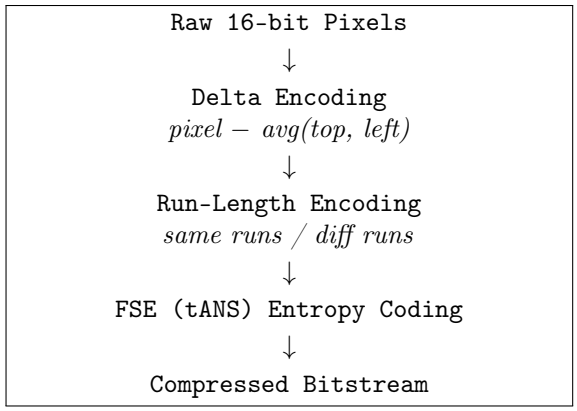

Figure 1: MIC compression pipeline for single-frame greyscale images.

3.1 Delta Encoding

The delta encoder computes the prediction residual for each pixel as:

$$r_{i,j} = x_{i,j} - \left\lfloor \frac{x_{i-1,j} + x_{i,j-1}}{2} \right\rfloor \tag{1}$$

where $x_{i,j}$ is the pixel at row i, column j. Boundary pixels use only the available neighbor (top for the first column, left for the first row) or the raw value for the top-left corner pixel.

For 16-bit images, residuals can span the range $[-65535, +65535]$. Rather than widening to 32 bits, MIC uses an *overflow delimiter* scheme derived from the image's effective bit depth $d = \lceil \log_2(\max(x) + 1) \rceil$:

- Residuals with $|r| \leq 2^{d-1} - 1$ are stored directly as uint16 values.
- Larger residuals are preceded by a delimiter value $2^d - 1$, followed by the raw pixel value.

This encoding adapts automatically to the actual bit depth of the image. For 8-bit images stored in 16-bit containers (common in MR), the threshold and delimiter operate within the 8-bit range, reducing the effective symbol alphabet and improving downstream entropy coding.

3.2 Run-Length Encoding

After delta coding, medical images produce long runs of identical residual values (especially zero) in smooth regions. MIC's RLE stage encodes the symbol stream as alternating *same runs* and *diff runs*:

- **Same run**: a count c followed by a single value repeated c times.
- **Diff run**: a count c followed by c distinct values.

A key design constraint is that the minimum run length is 3, which guarantees that the RLE output is *never larger* than the input. This eliminates the need for expansion detection or fallback paths.

The run headers use a midpoint protocol: counts $\leq$ `midCount` signal same runs; counts $>$ `midCount` signal diff runs with the actual count obtained by subtracting `midCount`.

3.3 FSE Entropy Coding

The final stage encodes the RLE output using table-based ANS (tANS), implemented as Finite State Entropy. MIC extends the standard FSE implementation to support up to 65 535 distinct symbols (versus the typical limit of 4 095), which is necessary for 16-bit medical images where delta residuals can span a wide range.

Encoding. The FSE encoder processes symbols in reverse order (last symbol first), building the compressed bitstream from the end. For each symbol, the encoder transitions between states using a pre-computed symbol transition table (`symbolTT`), emitting bits that encode the state transitions.

Decoding. The decoder reads bits forward, using a decode table (`decTable`) indexed by the current state to produce the next symbol and compute the next state. Each decode step requires only a table lookup, a bit read, and an addition—no divisions or multiplications.

Adaptive table sizing. MIC automatically adjusts the FSE table log from the default of 11 to 12 when the symbol density is sufficiently high (>128 distinct symbols with >32 data points each). This provides 4–7% better compression ratios on CR and mammography images where the residual distribution has a wider spread.

Canonical Huffman alternative. MIC also supports a canonical Huffman entropy backend. The Huffman encoder adaptively selects the symbol set and limits tree depth to $\leq$14 bits for compact codebook representation. It typically produces 1–3% smaller output than FSE but decodes more slowly due to the variable-length code lookup.

4 Container Formats

4.1 MIC1: Single Frame

The simplest format. A compressed frame is prefixed with a header containing the image dimensions and maximum pixel value, followed by the compressed bitstream.

4.2 MIC2: Multi-Frame

MIC2 supports multi-frame DICOM images such as breast tomosynthesis (DBT). The format consists of:

- A 20-byte header: magic bytes (`"MIC2"`), width, height, frame count, and pipeline flags.
- A frame offset table ($N \times 8$ bytes): each entry stores the byte offset and length of one compressed frame, enabling $O(1)$ random access.
- Concatenated compressed frame blobs.

MIC2 supports two compression modes:

Independent mode. Each frame is compressed independently using the spatial Delta+RLE+FSE pipeline. This allows random access to any frame without decompressing preceding frames.

Temporal mode. Frame 0 is compressed spatially. Subsequent frames store inter-frame residuals: $r_t = \text{frame}_t - \text{frame}_{t-1}$. These signed residuals are mapped to unsigned values via ZigZag encoding ($z = 0$ for $r = 0$; $z = 2r - 1$ for $r > 0$; $z = 2|r|$ for $r < 0$), then compressed with RLE+FSE (no spatial delta, since temporal residuals lack spatial correlation).

4.3 MIC3: Whole Slide Imaging

MIC3 is a tiled container for whole slide pathology images. It extends the pipeline to handle RGB images via a reversible color transform.

YCoCg-R color transform. RGB pixels are converted to Y (luminance), Co (orange chrominance), and Cg (green chrominance) using the reversible YCoCg-R transform [10]:

$$\text{Co} = R - B \tag{2}$$

$$t = B + \lfloor \text{Co}/2 \rfloor \tag{3}$$

$$\text{Cg} = G - t \tag{4}$$

$$Y = t + \lfloor \text{Cg}/2 \rfloor \tag{5}$$

For 8-bit RGB inputs, Y lies in $[0, 255]$, while Co and Cg lie in $[-255, 255]$. The chrominance planes are ZigZag-encoded to $[0, 510]$ before compression. Each plane (Y, Co, Cg) is then compressed independently through the Delta+RLE+FSE pipeline.

Tiled architecture. Images are divided into tiles (default 256×256 pixels). Each tile is compressed independently, enabling:

- $O(1)$ random access to any tile via the tile offset table.
- Parallel compression and decompression using a goroutine worker pool.
- Per-tile optimization: constant tiles (e.g., white background) compress to 15–17 bytes.

Pyramid levels. MIC3 stores multiple resolution levels, each at half the dimensions of the previous. Downsampling uses a 2×2 box filter applied in RGB space before color transform. Level descriptors in the header record the dimensions and tile grid of each level, enabling a viewer to select the appropriate resolution for the current zoom level.

Constant-plane optimization. When all pixels in a plane have identical values, the plane is stored as a single constant (3 bytes: mode byte + uint16 value) rather than running the full compression pipeline. For white background tiles (common in pathology), all three planes are

constant, yielding compression ratios exceeding 1 900×.

5 Implementation and Optimizations

MIC is implemented in Go (module `mic`, Go 1.22+). The implementation prioritizes decompression throughput, as the primary use case is real-time rendering of compressed images in viewers.

5.1 FSE Decode Loop Inlining

The innermost FSE decode loop in `decompress()` processes one symbol per iteration. State transitions are fully inlined with a local slice reference to the decode table, eliminating function call overhead and pointer indirections. This is the single most performance-critical code path in the codec.

5.2 Dual-Buffer Histogram

The symbol histogram function `countSimple()` uses two interleaved count arrays. Consecutive pixels are counted into alternating arrays, then merged. This reduces store-to-load forwarding stalls when consecutive pixels have similar values—a common pattern in medical images where adjacent pixels are highly correlated.

5.3 Branch-Free Delta Decompression

The delta decompressor uses four separate loops for corner, first-row, first-column, and interior pixels. The interior loop handles the vast majority of pixels ($> (w-1)(h-1)$ out of wh) without any per-pixel boundary checks, eliminating branch mispredictions.

5.4 RLE Fast Path

Same-value runs are the most common pattern after delta coding. The RLE decoder fast-paths same runs to return the cached value without touching the input slice, requiring only a counter decrement per output symbol.

Table 1: Test dataset: clinical DICOM images.

Image	Modality	Dimensions	Raw Size
MR	Brain MRI	256×256	0.13 MB
CT	CT scan	512×512	0.50 MB
CR	Comp. radiography	2140×1760	7.18 MB
XR	X-ray	2048×2577	10.1 MB
MG1	Mammography	2457×1996	9.35 MB
MG2	Mammography	2457×1996	9.35 MB
MG3	Mammography	4774×3064	27.3 MB
MG4	Mammography	4096×3328	26.0 MB

5.5 Dynamic Table Sizing

FSE encode and decode tables are sized to the actual symbol range rather than the theoretical maximum of 65 536. For 8-bit images, this reduces the working set from 512 KB to approximately 2 KB, substantially improving cache utilization.

5.6 Browser Decoders

MIC includes both a pure JavaScript ES module decoder (∼20 KB, zero dependencies) and a Go WebAssembly build. The JavaScript decoder achieves 10–30 M pixels/s in V8, sufficient for interactive viewing. Both decoders support MIC1, MIC2, and MIC3 formats, including multi-frame movie playback and RGB WSI tile rendering with window/level controls.

6 Experimental Results

6.1 Test Dataset

We evaluate MIC on eight clinical DICOM images spanning five modalities (Table 1). All images are 16-bit greyscale. Effective bit depth ranges from 8–12 bits (MR) to the full 16-bit range (CT).

6.2 Compression Ratios

Table 2 reports lossless compression ratios for the Delta+RLE+FSE pipeline.

Mammography images achieve the highest ratios (up to 8.57×) due to large smooth breast tissue regions that produce near-zero residuals after delta coding. CT images with full 16-bit dynamic range achieve lower ratios (2.24×) because

Table 2: Lossless compression ratios (Delta+RLE+FSE).

Image	Raw Size	Compressed	Ratio
MR	0.13 MB	0.053 MB	2.35×
CT	0.50 MB	0.22 MB	2.24×
CR	7.18 MB	1.98 MB	3.63×
XR	10.1 MB	5.79 MB	1.74×
MG1	9.35 MB	1.09 MB	8.57×
MG2	9.35 MB	1.09 MB	8.55×
MG3	27.3 MB	12.2 MB	2.24×
MG4	26.0 MB	7.49 MB	3.47×

Table 3: Effect of adaptive tableLog (11→12).

Image	Before	After	Gain
CR	3.47×	3.63×	+4.4%
MG1	7.99×	8.57×	+7.1%
MG2	7.98×	8.55×	+7.1%

the wider value range increases residual entropy. X-ray images achieve the lowest ratios (1.74×) due to high-frequency noise.

Adaptive tableLog improvement. Automatically increasing the FSE table log from 11 to 12 for images with high symbol density yields measurable gains (Table 3).

6.3 Decompression Throughput

Table 4 reports decompression throughput across four hardware platforms. The primary benchmark metric is MB/s of raw (uncompressed) pixel data produced. Table 5 expresses the same results as frames per second (FPS) on the AWS c7g.metal platform.

Key observations:

- **Peak throughput of 16.4 GB/s** is achieved on MG1 (mammography) on the 64-core ARM64 platform, approaching DRAM bandwidth limits.
- Throughput scales roughly linearly with core count: the 32-core ARM64 instance achieves approximately half the throughput of the 64-core instance.
- ARM64 platforms outperform x86 at equivalent core counts, likely due to wider memory buses on AWS Graviton3 instances.
- Larger images achieve higher throughput due to better amortization of fixed overhead (header parsing, table construction).
- RAM bandwidth is the primary bottleneck, not CPU speed. DDR5 platforms consistently outperform DDR4 platforms.

6.4 Multi-Frame Results

Table 6 reports compression results for a 69-frame breast tomosynthesis study (2457×1890, 10-bit depth).

Independent mode slightly outperforms temporal mode on this dataset because mammography images have strong spatial correlation (the spatial predictor is very effective) but only moderate inter-frame correlation (each slice images a different plane of breast tissue). Temporal mode may be advantageous for sequences with higher inter-frame correlation, such as cardiac cine MRI or fluoroscopy.

6.5 WSI Compression Results

Table 7 reports compression results for whole slide image tiles.

The YCoCg-R color transform is particularly effective for pathology images. H&E-stained tissue produces smooth color gradients in the luminance and chrominance planes, yielding strong spatial prediction. Background tiles (slide glass regions) are nearly constant and compress to 15–17 bytes total via the constant-plane optimization.

7 Comparison with Wavelet-Based Compression

To evaluate whether a more sophisticated decorrelation stage could improve MIC, we implemented a Le Gall 5/3 integer wavelet transform [11]—the same transform used in JPEG 2000 lossless mode—as an alternative to delta encoding.

Table 4: Decompression throughput (MB/s of raw pixel data) across hardware platforms.

Platform	MR	CT	CR	XR	MG1	MG2	MG3	MG4
AWS c7g.metal (ARM64, 64c)	2 282	4 433	8 527	9 411	**16 387**	16 023	8 044	15 213
AWS c7g.8xl (ARM64, 32c)	1 524	2 186	4 290	4 562	8 901	7 879	4 455	7 132
AWS c7i.8xl (x86, 32c, Xeon 8488C)	1 142	1 208	3 172	3 269	5 220	5 124	3 468	4 964
Mac Studio (M2 Max, 12c)	1 054	1 121	2 089	2 101	3 666	3 659	2 239	3 188

Table 5: Decompression in frames per second (FPS) on AWS c7g.metal.

Image	FPS	MB/s
MR (256×256)	17 411	2 282
CT (512×512)	8 455	4 433
CR (2140×1760)	1 132	8 527
XR (2048×2577)	892	9 411
MG1 (2457×1996)	1 671	16 387
MG2 (2457×1996)	1 634	16 023
MG3 (4774×3064)	281	8 044
MG4 (4096×3328)	558	15 213

Table 6: Multi-frame compression (69-frame DBT, 614 MB raw).

Mode	Compressed	Ratio
Independent (spatial)	46.1 MB	13.3×
Temporal (inter-frame)	47.5 MB	12.9×

7.1 Wavelet Implementation

The 5/3 lifting scheme applies predict and update steps:

$$d[i] = x[2i+1] - \left\lfloor \frac{x[2i] + x[2i+2]}{2} \right\rfloor \tag{6}$$

$$s[i] = x[2i] + \left\lfloor \frac{d[i-1] + d[i] + 2}{4} \right\rfloor \tag{7}$$

The 2D transform applies 1D transforms to all rows, then all columns. Wavelet coefficients are ZigZag-encoded to unsigned values, with an overflow escape for coefficients exceeding the uint16 range.

We evaluated two wavelet pipelines:

- Wavelet+FSE: wavelet transform → FSE (no RLE).
- Wavelet+RLE+FSE: wavelet transform → RLE → FSE.

Table 7: WSI tile compression ratios (MIC3, YCoCg-R + Delta+RLE+FSE).

Tile Type	Ratio	Notes
White background	1 946×	Near-zero entropy
Dense tissue (H&E)	4.4×	Smooth staining gradients
Gradient	5.4×	Excellent spatial correlation
Mixed slide	4–8×	Weighted avg. of tile types

Table 8: Compression ratio: Delta+RLE+FSE vs. wavelet pipelines.

Image	Delta+ RLE+FSE	Wavelet +FSE	Wavelet+ RLE+FSE
MR	**2.35**×	2.09×	2.09×
CT	**2.24**×	1.48×	1.48×
CR	**3.63**×	2.59×	2.59×
XR	**1.74**×	1.53×	1.53×
MG1	**8.57**×	4.91×	7.28×
MG2	**8.55**×	4.90×	7.27×
MG3	**2.24**×	1.90×	1.93×
MG4	**3.47**×	2.63×	3.11×

7.2 Compression Ratio Comparison

Table 8 compares compression ratios. Delta+RLE+FSE achieves the best ratio on *every* modality.

7.3 Decompression Speed Comparison

Table 9 compares single-threaded decompression speed. Delta+RLE+FSE is faster on all large images (≥2K pixels); the wavelet pipeline wins only on small images (MR, CT) where the overhead of the wavelet inverse is amortized.

Table 9: Decompression speed (MB/s): Delta+RLE+FSE vs. wavelet.

Image	Delta+ RLE+FSE	Wavelet +FSE	Wavelet+ RLE+FSE
MR	116	**146**	122
CT	165	**168**	142
CR	**543**	418	371
XR	**605**	576	486
MG1	**1 530**	592	680
MG2	**1 493**	618	644
MG3	**606**	387	352
MG4	**1 054**	480	579

7.4 Analysis

Delta encoding outperforms the wavelet transform for lossless medical image compression for three reasons:

1. **The linear predictor is near-optimal.** Medical images have smooth intensity gradients and large homogeneous regions. The average-of-neighbors predictor produces residuals tightly clustered around zero. RLE then compresses long runs of identical residuals directly.
2. **The wavelet update step hurts.** The 5/3 update step reintroduces correlation in the low-pass band to preserve the signal mean (needed for perfect reconstruction). For images where delta residuals are already near-zero, this provides no net benefit. For full-range CT images, the update step forces escape encoding when low-pass coefficients overflow uint16, inflating the compressed stream.
3. **Delta decompression uses half the memory bandwidth.** The wavelet inverse requires two full passes over the pixel buffer (horizontal and vertical), each touching every element twice. It also operates on int32 values (4 bytes/sample) versus uint16 (2 bytes/sample) for delta. For large mammography images at high parallelism, this doubles the memory bandwidth requirement, which is the binding constraint.

The wavelet transform remains valuable for two use cases outside MIC's current scope: *lossy compression* (where wavelet coefficients can be quantized by subband) and *progressive/multi-resolution delivery* (where the LL subband is a valid downscaled image at each decomposition level).

8 Discussion

Design philosophy. MIC deliberately avoids complex prediction models (context modeling, Markov predictors) in favor of a simple three-stage pipeline where each stage has $O(n)$ complexity and minimal branching. This design sacrifices a small amount of compression ratio (compared to JPEG 2000) in exchange for dramatically higher decompression throughput. For clinical workflows where images are compressed once and decompressed many times (archival → viewing), this tradeoff is strongly favorable.

Bit-depth adaptivity. A notable feature of MIC is its automatic adaptation to the effective bit depth of the image. All thresholds and delimiters are derived from the actual maximum pixel value via $\lceil \log_2(\max + 1) \rceil$, eliminating the need for separate 8-bit and 16-bit code paths. This simplifies the implementation and ensures optimal performance across the full range of DICOM bit depths (8–16 bits).

Limitations. MIC does not support lossy compression, progressive decoding, or region-of-interest coding for greyscale images. These capabilities are available in JPEG 2000 and may be important for bandwidth-limited telemedicine applications. The wavelet transform, already implemented and benchmarked, provides a natural foundation for a future lossy mode.

For WSI images, MIC3's tiled architecture provides tile-level random access and pyramid-level multi-resolution support, addressing the region-of-interest and progressive delivery requirements specific to digital pathology.

Comparison with HTJ2K. High-Throughput JPEG 2000 (HTJ2K) [3] represents the closest point of comparison for MIC. HTJ2K achieves

comparable compression ratios to JPEG 2000 with significantly faster decoding. MIC achieves comparable or slightly lower compression ratios than JPEG 2000/HTJ2K but with substantially higher decompression throughput, particularly on ARM64 platforms with wide memory buses. A detailed quantitative comparison with HTJ2K is an important direction for future work.

9 Conclusion

We have presented MIC, a lossless medical image codec that achieves compression ratios of 1.7×–8.6× on greyscale DICOM images and 3–5× on RGB pathology images, with decompression throughput up to 16 GB/s. The codec's three-stage pipeline (delta prediction, run-length encoding, FSE entropy coding) is designed for simplicity and speed: each stage processes the data in a single pass with minimal branching, enabling decompression performance that scales with memory bandwidth.

MIC supports three container formats for different clinical use cases: single-frame (MIC1), multi-frame with temporal prediction (MIC2), and tiled whole slide imaging with pyramid levels (MIC3). The implementation includes browser-based JavaScript and WebAssembly decoders for web viewing applications.

Our evaluation demonstrates that the simple delta+RLE+FSE pipeline strictly dominates a wavelet-based alternative on all tested DICOM modalities, in both compression ratio and decompression speed. This result suggests that for lossless medical image compression, simple linear prediction combined with fast entropy coding is more effective than the sophisticated wavelet transforms used in JPEG 2000.

MIC is open-source and available at https://github.com/pappuks/medical-image-codec.

Availability. Source code, test data, and benchmark scripts are available under an open-source license at the URL above.

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